

Research State Briefing

Recorded hospitalisations can fall without cardiovascular health improving.

First Auto Research run: epistemic reconstruction

Current boundary: descriptive counts and exploratory model sensitivity.
COVID mechanisms and the causal target remain unresolved.

Sources: C02, C07, C08, C09 · source links and limits in notes/register

The outcome is a recorded first event

AVAILABLE IN THE PUBLIC RECORD

CHD and HF · territory-month
T2D and/or hypertension population
January 2013–December 2023
132 months per outcome

First hospitalisation after a first diagnosis record.
Query construction remains to be confirmed.

UNRESOLVED OR NOT DELIVERED

Admission cause and cohort person-time
Entry/exit, washout and deduplication
Inpatient/date/ICD definitions
Labs, infection, vaccine and serology
Stroke series

A receipt is evidence of delivery.
It is not a validated data dictionary.

Sources: C02, C13, C14 · source links and limits in notes/register

Five transitions explain the current state

- 17 JUL** Admission reasons absent → AMI framing demoted; monthly aggregates planned.
- AUG** CHD/HF delivered → amended core; no protected primary; daily recovery fails gates.
- 27 AUG** 6-df “crown” preserves preferred intervals → selection history, not corroboration.
- 10 SEP** Hogan reportedly proposes COVID direction → same tables acquire a new framing.
- 12–13 SEP** Candidate repairs narrow the narrative → manuscript authority remains conflicted.

Sources: C01, C03, C10, C12, C05, C06 · source links and limits in notes/register

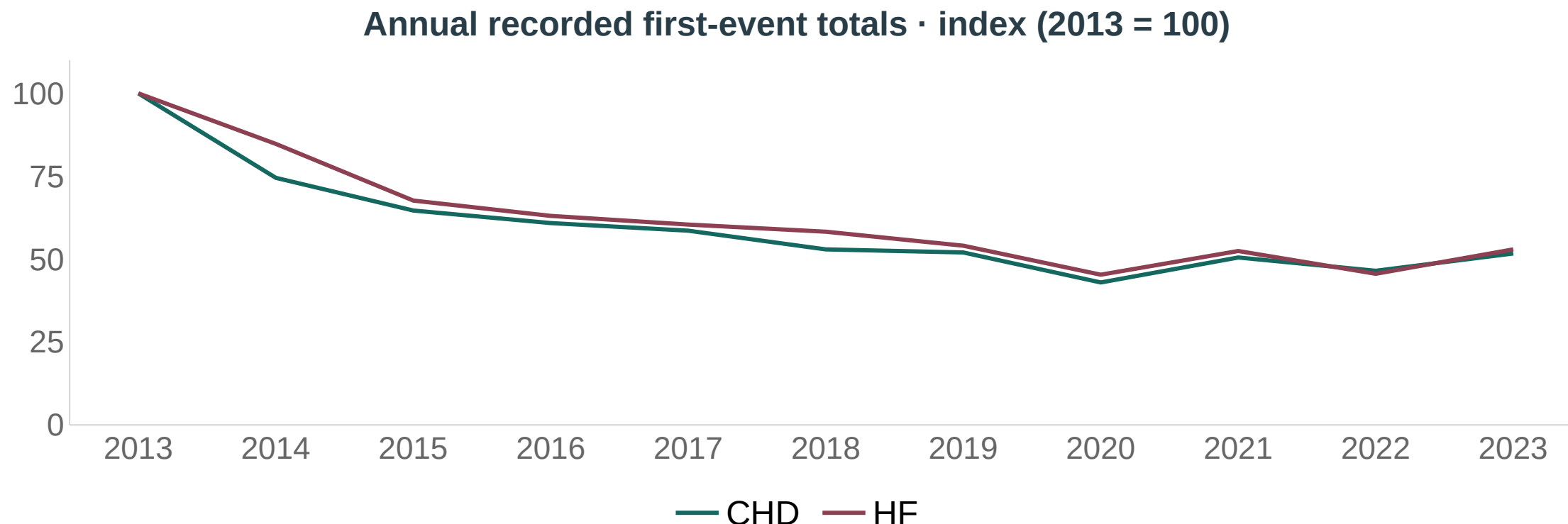
Three objects require three interpretations

Object	What is measured	What it cannot establish
Recorded burden	Annual totals of first events in this extraction	Incidence, cause of admission or improved health
Existing model parameter	Weather count ratio conditional on month, time smooth and days offset	A COVID treatment effect or a tested window difference
Proposed causal target	Pandemic-era change in health, service use or recorded events	No agreed intervention, counterfactual or identified target yet

84 months are inside 132. Subsetting changes spline knots and exposure support.
Five official days is a reporting scale; COVID-phase adjustment is not a third window.

Sources: C02, C03, C07, C08 · source links and limits in notes/register

Most endpoint decline occurred before 2020



2019 was already near half of 2013. A 2020 trough was followed by partial recovery. This weakens a COVID-only decade explanation; it does not identify either component.

Sources: C15 · source links and limits in notes/register

Point estimates move; a difference was not tested

Count ratio per five days	Nested 84 months	Full 132 months
HF · cold days	1.113 (1.053–1.176)	1.073 (1.006–1.144)
CHD · cold days	1.036 (1.007–1.067)	0.995 (0.949–1.043)
CHD · hot nights	1.011 (0.991–1.032)	1.022 (1.002–1.042)

Existing Newey–West lag-6 intervals. Full-window BH q-values all exceed 0.19; nested fits are outside that multiplicity family.

Overlapping samples + re-knotted trend ≠ independent pre/post contrast.

Sources: C07, C09, C16, C17 · source links and limits in notes/register

Keep failures at the scope actually tested

Method admission failed

Synthetic daily-recovery calibration: Type I error reached 0.150; coverage fell to 0.840. A failure of the tested implementation/scenarios, not a universal aggregation theorem.

Discovery and mechanism claims remain unlicensed

No protected thermal discovery does not prove no effect. Missing clinical measures do not falsify every biological explanation. The 6-df crown is result-dependent.

Checks need semantic inspection

Legacy auditor: one-check pass on the wrong schema. Dedicated rails verify display, sources and document contracts; numerical agreement does not identify causes.

Sources: C09, C10, C12, C18, C20 · source links and limits in notes/register

Four rival families need different evidence

Rival family	Pattern that would help distinguish it	Evidence needed
Entry / risk set / extraction	Movement follows eligibility, look-back or cohort entry/exit	Query provenance; dynamic denominators
Utilisation / competing death	Fewer contacts with delayed care or increased out-of-hospital death	Service, diagnosis and mortality linkage
Biology / co-exposures	Clinical risk or infection/pollution changes beyond recording effects	Specified clinical measures and comparison
Model / support / uncertainty	Movement follows knots, support, influence or covariance choice	Bounded, prespecified diagnostics

These explanations can coexist. Current counts do not discriminate them.

Sources: C13, C14, C19, C08 · source links and limits in notes/register

Five questions change the next research decision

1 · WHAT IS A FIRST EVENT?

Roro/DAE: confirm query, dates, entry/exit, washout, ICD and inpatient definition.

2 · WHAT CHANGED IN THE PANDEMIC ERA?

Agent + Hogan: specify a deviation beyond the earlier decline and its counterfactual.

3 · CAN HEALTH AND OBSERVATION BE SEPARATED?

Roro/Bishai: assess risk-set, service, diagnosis and mortality evidence feasibility.

4 · WHY DO COEFFICIENTS MOVE?

Agent: separate spline basis, support, influence, uncertainty and effect modification.

5 · WHICH TARGET AND DOCUMENT DOES THE TEAM ENDORSE?

Bob/Hogan/Bishai: resolve authority; choose narrowing, evidence acquisition, pivot or stop.

Sources: C02, C05, C06, C07, C13, C14 · source links and limits in notes/register

Phase 2 begins with discriminating predictions

NOW · Retain the recorded-count and sensitivity evidence

Freeze this reconstruction as a reviewable starting point. Resolve event construction, scientific target and document authority before another manuscript cycle.

IF EVIDENCE IS FEASIBLE · Design severe tests

Use a causal graph; state rival predictions, meaningful contrasts and rejection rules. Ask what the model fails to explain and whether the test can detect that failure.

IF RIVALS CANNOT BE SEPARATED · Narrow, pivot or stop

A progressive move gains new independent predictions. Rescuing a headline with another preferred fit is not evidence of progress. Gate 3 remains human-owned.

Sources: C08, C09, C13, C14, C19 · source links and limits in notes/register